

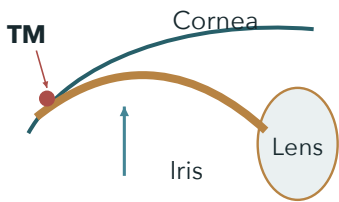
Secondary and developmental outflow

TYPE B | Decide whether tissue is crowded, blocked or malformed.

Peripheral crowding has different causes

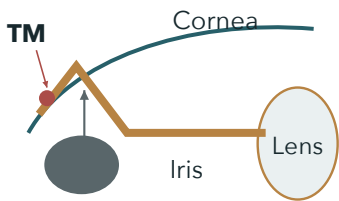
Three angle anatomies

PUPILLARY BLOCK



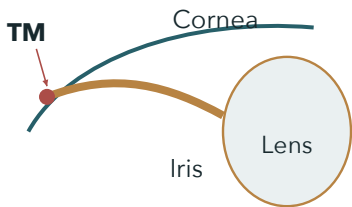
Pressure difference bows iris forward

PLATEAU IRIS



Ciliary process supports peripheral iris; centre flat

PHACOMORPHIC CROWDING



Enlarged lens shallows the chamber

A flat central iris and persistent peripheral crowding after a patent iris opening favour plateau iris. The ciliary processes are the key support.

Same contact, different initiating lesion

Pattern	Primary driver
Pupillary block	Posterior chamber pressure bows peripheral iris forward
Plateau iris	Anterior ciliary processes elevate the peripheral iris
Phacomorphic crowding	Enlarged lens displaces iris and shallows the chamber

Developmental outflow failure

Malformed drainage tissue raises IOP in congenital glaucoma. The infant globe is relatively distensible: enlargement, corneal oedema and Descemet breaks can accompany pressure-related stretching.

An open angle can still drain poorly

Secondary obstruction map OPEN ANGLE, IMPAIRED FLOW

Steroid response

A diagram illustrating the steroid response. It shows a series of vertical blue lines representing drainage tissue. An arrow points from left to right through these lines. The lines are slightly thicker and more closely spaced than in a normal state, indicating an increase in resistance to fluid outflow.

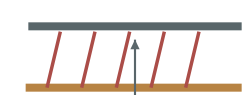
Drainage tissue changes
raise resistance

Phacolytic protein + cells

A diagram illustrating phacolytic protein and cells. It shows a series of vertical blue lines representing drainage tissue. Three yellow circles, representing lens material or macrophages, are shown clogging the spaces between the lines. A small red dot is also visible above one of the yellow circles.

Lens material and
macrophages clog spaces

NEOVASCULAR MEMBRANE

A diagram illustrating the formation of a neovascular membrane. It shows a horizontal line representing the retina. Below it, several red lines represent the angle membrane. An arrow points upwards from the angle membrane towards the retina, indicating the growth of a neovascular membrane. The text 'Retinal ischaemia -> VEGF' and 'Angle membrane -> contraction' is present.

Retinal ischaemia -> VEGF
Angle membrane -> contraction

The material and its site matter. Neovascular disease can start by covering drainage tissue and progress to contraction with synechial closure.

Cause → obstruction

Cause	High-yield mechanism
Steroid response	Altered drainage tissues increase conventional resistance
Phacolytic glaucoma	Lens protein plus macrophages obstruct trabecular spaces
Neovascular glaucoma	Ischaemia → VEGF → iris/angle fibrovascular membrane
Congenital glaucoma	Developmentally abnormal outflow tissue

Structure is not a pressure label

Glaucoma injures retinal ganglion-cell axons at the optic nerve head. High IOP increases risk, but normal-range IOP does not exclude glaucomatous damage; assess the rim, nerve fibre layer and corresponding field.